

21/04/2026

Dr Thomas Briggs
7/377 Cavendish Rd
COORPAROO QLD 4151
hello@focusperio.com.au

Name: Karoly Szabo
D.O.B: 30/03/1957
Phone: 0414 4040 96
Email: kandkszabo@gmail.com

Dear Dr Briggs,

Thank you for referring Karl for assessment of his failing implant-supported bridgework.

Mr Szabo presented with complaints of recurrent food impaction and difficulty cleaning under the upper right bridge. This has led to recurrent infections requiring antibiotics on one to two occasions. The existing maxillary cement-retained implant bridge was placed in approx 2017 and is supported by 10 implants. His medical history is significant for Prolia (denosumab), with his most recent injection being on the day of the consultation. (20/04/2026)

Intraoral examination showed a maxillary cement-retained implant bridge in situ, with no mobility noted on attempted removal. Soft tissues showed evidence of infection around the 14/16 implants. There is significant radiographic bone loss to the apex of the implants at sites 16 and 14, with one of these implants appearing to have no remaining bone support.

Diagnosis: Failing cemented implant-supported bridgework with two failing implants (16 and 14).

I discussed the diagnosis and complexity of the situation with Karl, particularly the difficulty of removing a 10-unit cement-retained prosthesis. We discussed the risk of medication-related osteonecrosis of the jaw (MRONJ) associated with Prolia and surgical intervention.

A conservative, staged approach was recommended. The initial plan involves sectioning the bridge distal to implant 13 to remove the failing implants (16, 14) and the attached bridge section. The remaining bridgework from 13-26 would be maintained. The implant at site 17 could potentially be sectioned from the removed bridge section and re-restored as a single crown. This approach aims to eliminate the infection while preserving as much of the existing functional prosthesis as possible. Long-term options, including a removable prosthesis, were mentioned and can be considered after the initial healing and stabilisation phase.

Patient Education & Preventative Advice: I have advised Mr Szabo to attempt to obtain the brand and size details of his implants from the original provider, as this will be critical for treatment planning and component availability in Australia.

There is a good chance that the implants will be removed with the bridge if sectioned. However, it may require further surgical intervention. I have also advised that the timing of surgery in relation to his Prolia injection cycle may need consideration, although leaving the active infection also carries risks. I can organise to complete the bridge section removal on a day appropriate that you can also see him to manage the implants.

Thank you for your referral and collaboration in the care of Karoly Szabo. Please feel free to contact me should you require any further details or discussion regarding this case.

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DR. JUSTIN BOULTON
PROSTHODONTIST

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DR. CASEY WALSH
PROSTHODONTIST

BDS, GDipDlplant, DClindent (Pros)

Kindest Regards,

A handwritten signature in dark ink, appearing to read 'Walsh', is positioned below the text 'Kindest Regards,'.

Dr Casey Walsh
BDS, GDipDlplant, DClindent (Pros)

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